BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. National Cancer Control Programme (NCCP) remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment. After stopping: six-monthly to 24 months; after anthracyclines in early disease, yearly to 5 years or longer if needed.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Additional tests if needed	Team / companion-drug plan	Team / companion-drug plan	Repeat according to the team / companion-drug plan; no fixed universal interval in this NCCP card.
electrocardiogram (ECG) / rhythm	Baseline ECG in EMA cardiac assessment; it does not replace echo.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Document the disease sites and imaging purpose	Assess symptoms at visits	Scan only if due / indicated	Metastatic: often near a response review; exact date agreed	Early: breast surveillance, symptom-led additional tests. Metastatic: agreed response scans, commonly every 2–4 months; markers are supplementary.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	If endocrine / ovarian suppression is planned: assess need	Not a routine trastuzumab or T-DM1 test alone	Timing depends on risk / result	Separate from the echo schedule	DEXA and repeat timing depend on endocrine treatment, fracture risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00200 v8 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · cardiac assessment and formulation

THE WHOLE TREATMENT JOURNEY

Trastuzumab intravenous (IV) · every 3 weeks: the whole pathway

HER2-positive early or metastatic breast cancer

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00200	Confirm setting, prior doses and formulation	IV trastuzumab every 3 weeks	Review benefit, symptoms and heart function	Early: complete one year total. Metastatic: ongoing review.	Early: completed year if applicable. Metastatic: review whether to continue or change.
Surgery EARLY	Early: surgery may precede or follow initial systemic therapy	Neoadjuvant route: reassess before planned surgery	multidisciplinary team (MDT) reviews recovery and pathology	Early: continue prescribed postoperative HER2 plan	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Early: usually after chemotherapy; coordinate surgery	May overlap trastuzumab continuation if agreed	Early: radiotherapy (RT) may overlap trastuzumab continuation, when agreed.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00200	Baseline FBC, renal, liver	Repeat according to the team / companion regimen	According to the team / companion regimen	Repeat according to the team / companion regimen	Follow abnormalities and the next treatment plan
Heart checks NCCP 00200 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail.
Scans / markers EARLY · METASTATIC · MARKERS	Define baseline sites and scan purpose	Review symptoms at each visit	Metastatic response imaging if due; not an automatic early-breast computed tomography (CT)	Individualised response or breast surveillance plan	Early: breast surveillance, symptom-led additional tests. Metastatic: agreed response scans, commonly every 2–4 months; markers are supplementary.

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Endocrine / bone health EARLY · BONE	Check hormone receptor status and endocrine indication	Endocrine timing coordinated with the wider plan	DEXA if indicated by endocrine / fracture risk	May overlap trastuzumab when clinically appropriate	Longer endocrine and bone follow-up can outlast HER2 treatment

What each check is for

1

Blood tests

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here)

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

Sources: NCCP 00200

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Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

Sources: NCCP 00200 · SMPC

3

Cancer imaging and markers

Early disease: planned breast surveillance; no routine CT or tumour-marker screening for recurrence without symptoms. Metastatic disease: response imaging on an agreed schedule, commonly every 2–4 months; earlier for new concerns.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

Sources: EARLY · METASTATIC · MARKERS

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Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00200

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-21 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.14